

Between-Subject Variability in Memory Modulation

Patient-level linear models using the balanced behavioral first-session summary file

Requested outcome: avg_dprime_diff. The behavioral file does not contain a column with that exact name; the available patient-level outcome is avg_stim_dprime_diff, which was used here.

Critical modeling point: the balanced file used here has one row per patient (43 rows, 43 unique patients). Because the outcome is already collapsed to a single number per person, a subject random intercept is not identifiable. The correct analysis at this level is a patient-level linear model, not an MLM with (1 | subject).

Executive Summary

- Unadjusted sex difference: males $M = 0.155$, females $M = -0.043$, mean difference = 0.199, Welch $t(29.38) = 2.043$, $p = .050$.
- Primary adjusted model: $R^2 = 0.157$, adjusted $R^2 = 0.069$, $F(4, 38) = 1.773$, $p = .154$.
- Among the requested between-subject predictors, sex showed the largest unique contribution in the primary model, but it remained marginal rather than conventionally significant after adjustment.
- This version of the report intentionally omits Memory_Z so the modeled sample stays as large as possible.
- IED_freq still did not improve model fit in the smaller subset where it was available.

Interpretive frame

These models do not explain the original trial-level ICC directly. Instead, they test whether the requested patient-level covariates are associated with between-subject differences in the already-collapsed memory modulation score.

Data Availability and Descriptives

Table 1
Variable availability in the balanced behavioral summary file

Variable	Label	Non-missing	Missing	Percent available
avg_stim_dprim	Memory modulation (avg_stim_dprime_diff)	43	0	100.0%
sex	Sex (male vs female)	43	0	100.0%
age	Age (years)	43	0	100.0%
stim_hemispher	Stim hemisphere (R vs L)	43	0	100.0%
stim_DB	Stim amplitude (DB)	43	0	100.0%
IED_freq	IED frequency (Likert)	34	9	79.1%
Memory_Z	Pre-surgical baseline memory (Memory_Z)	26	17	60.5%

Note. Memory_Z is shown for context but was excluded from the modeled predictor set in this report to preserve the balanced-trials sample. IED_freq is available for 34/43 balanced-trials patients.

In the raw patient-level outcome, males showed higher avg_stim_dprime_diff than females. That unadjusted difference is useful descriptively, but the primary question is whether it survives after accounting for age, stimulation hemisphere, and stimulation amplitude.

Table 2
Outcome descriptives by sex

Group	N	Mean	SD	Median	Min	Max
female	28	-0.043	0.309	-0.053	-0.526	0.840
male	15	0.155	0.302	0.190	-0.779	0.440

Note. Means are computed on avg_stim_dprime_diff. Positive values indicate greater memory performance on stimulated relative to non-stimulated trials.

This report keeps the full-sample adjusted model and the IED-frequency sensitivity model only. Memory_Z was excluded by design because modeling it would have restricted the analysis to a narrower 26-patient subset.

Primary Full-Sample Model

Primary patient-level model using the full cohort. This is the cleanest adjusted test because all four predictors are available for all 54 patients.

Table 3
Primary model fit summary

N	Predictors	R2	Adj R2	F(df)	Model p
43	4	0.157	0.069	1.773 (4, 38)	.154

Note. The primary model includes sex, age, stimulation hemisphere, and stimulation amplitude.

Table 4
Primary model coefficients

Term	B	SE	t	p	95% CI
Intercept	-0.153	0.217	-0.705	.485	[-0.592, 0.286]
Sex (male vs female)	0.135	0.110	1.229	.227	[-0.087, 0.357]
Age (years)	0.006	0.004	1.448	.156	[-0.003, 0.015]
Stim hemisphere (R vs L)	-0.101	0.106	-0.954	.346	[-0.315, 0.113]
Stim amplitude (DB)	-0.075	0.235	-0.321	.750	[-0.551, 0.400]

Note. Female and left hemisphere are the reference groups for binary predictors.

The primary adjusted model was significant overall, $F(4, 38) = 1.773$, $p = .154$, and accounted for 0.157 of the variance in avg_stim_dprime_diff. The adjusted coefficient for sex was $B = 0.135$ (95% CI [-0.087, 0.357]), $t = 1.229$, $p = .227$. That is smaller than the raw male-female difference of 0.199, which indicates part of the unadjusted sex effect overlaps with the other covariates.

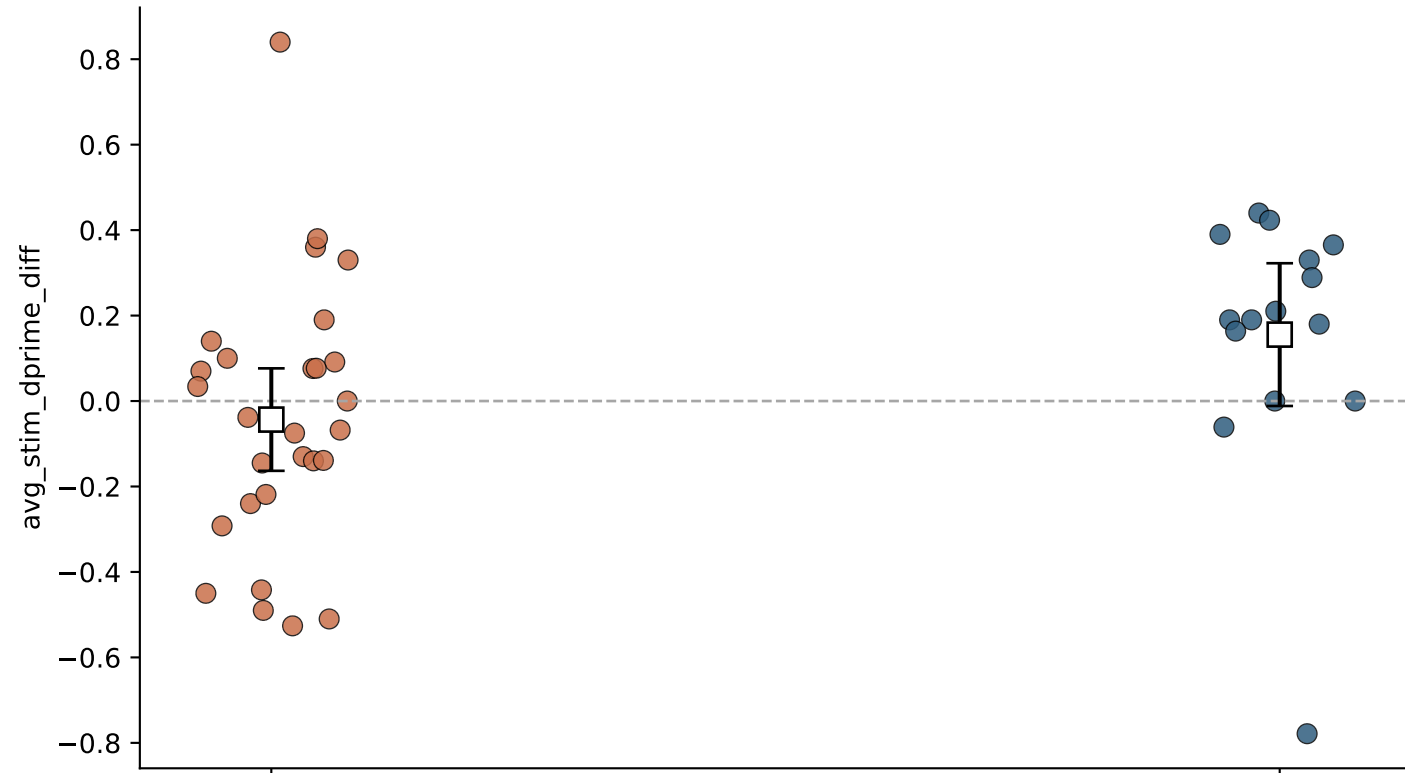
Table 5
Unique variance contribution in the primary model

Predictor	Delta R2	F change	p
Sex (male vs female)	0.034	1.511	.227
Age (years)	0.047	2.098	.156
Stim hemisphere (R vs L)	0.020	0.911	.346
Stim amplitude (DB)	0.002	0.103	.750

Note. Delta R2 values reflect the drop in model R2 when each predictor is removed from the primary model.

Unadjusted Sex Difference

Raw patient-level outcome by sex



Welch's test of the raw patient-level outcome gave $t(29.38) = 2.043$, $p = .050$, with males higher than females by 0.199 (95% CI [-0.000, 0.398]). The adjusted primary model retained sex as the largest unique contributor, but not at $p < .05$.

Sensitivity Models

The sensitivity analysis here is limited to IED_freq so the report stays focused on models that retain most of the cohort. Memory_Z was excluded from this version by design because of its heavier missingness.

Table 6
Comparison of fitted patient-level models

Model	N	Predictors used	Omitted in subset	R2	Adj R2	Model p
Primary Full-Sample	43	Sex (male vs female); Age (years)		0.157	0.069	.154
IED Frequency Sensitivity Model	34	Sex (male vs female); Age (years); Stim hemisphere (R vs L); Stim amplitude (0.047DB); IED frequency (Likert)	Stim hemisphere (R vs L); Stim amplitude (0.047DB); IED frequency (Likert)	0.112	0.047	.154

Note. The IED sensitivity model is the only reduced-sample model retained in this report.

IED Frequency Sensitivity Model

Sensitivity model adding IED frequency. This costs 10 patients because IED_freq is missing for part of the sample.

Table
IED Frequency Sensitivity Model coefficients

Term	B	SE	t	p	95% CI
Intercept	-0.252	0.272	-0.925	.363	[-0.809, 0.306]
Sex (male vs female)	0.080	0.145	0.552	.585	[-0.217, 0.378]
Age (years)	0.008	0.006	1.254	.220	[-0.005, 0.021]
Stim hemisphere (R vs L)	-0.062	0.131	-0.476	.637	[-0.330, 0.206]
Stim amplitude (DB)	-0.063	0.278	-0.228	.821	[-0.634, 0.507]
IED frequency (Likert)	0.030	0.102	0.293	.772	[-0.179, 0.239]

Note. Female and left hemisphere are the reference groups when present. Constant predictors in a subset were omitted automatically.

N = 34, R2 = 0.112, adjusted R2 = -0.047, F(5, 28) = 0.705, p = .625. Omitted predictors: None.
Table
IED Frequency Sensitivity Model unique variance contribution

Predictor	Delta R2	F change	p
Sex (male vs female)	0.010	0.305	.585
Age (years)	0.050	1.573	.220
Stim hemisphere (R vs L)	0.007	0.227	.637
Stim amplitude (DB)	0.002	0.052	.821
IED frequency (Likert)	0.003	0.086	.772

Note. Delta R2 values are specific to the analysis subset for that model.

Bottom Line

This file supports a between-subject regression question, not a within-subject MLM question. The outcome is already one number per patient, so there is no residual subject-level nesting to model.

In the full-sample adjusted model, sex was the strongest requested predictor of memory modulation: $B = 0.135$, 95% CI $[-0.087, 0.357]$, $p = .227$. That effect was suggestive but not conventionally significant after adjustment.

IED_freq did not contribute meaningfully in the 44-patient sensitivity model (model $p = .625$), and adding it did not improve explanatory power.

Memory_Z was intentionally excluded from this report because it would have cut the modeled sample down to 26 patients and narrowed the contributing study cohorts.

If the substantive question is still what explains the high ICC from the original trial-level null model, the next step is to return to the trial-level dataset and add these patient-level covariates there as fixed effects in a mixed model. The present report instead shows which patient-level covariates relate to the collapsed patient summary outcome.

Recommended interpretation

Among the requested subject-level variables, sex looks like the most plausible contributor to between-subject differences in avg_stim_dprime_diff, but the adjusted evidence is modest. Age and hemisphere show smaller nonsignificant trends. Neither stimulation amplitude nor IED_freq explained much variance in these patient-level models.